

In this article, I describe four interrelated analytical concepts useful for studying the discursive practices of professional writers: intertextuality, interdiscursivity, genre systems, and recontextualization. Drawing on structuration theory and neo-Vygotskian activity theory to provide a framework for the above concepts, I present three theoretical assumptions: (a) genre systems play an intermediate role between institutional structural properties and individual communicative action, (b) a central means for identifying texts in a genre system is their intertextual activity, and (c) the concept of "genre systems" enables the analyst to foreground the discursively salient components of human activity systems. An elaboration of each of these assumptions is followed by an illustration of genre systems at work in one psychotherapist's session notes and the process I call rhetorical recontextualization.

Genre Systems at Work

DSM-IV and Rhetorical Recontextualization in Psychotherapy Paperwork

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Within the past few decades, new conceptions of genre have emerged in a number of disciplines, such as functional/systemic linguistics, anthropological linguistics (and linguistic anthropology), applied linguistics, rhetoric, literary theory, composition studies, ethnography of communication, and sociology of knowledge. The influence of the theoretical work of Bakhtin (1981, 1986) on speech genres and intertextuality has been pervasive in these fields, leading to what many commentators call the "communicative turn" in genre analysis. One effect of this communicative turn in recent research has been a

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burgeoning interest in the intertextual and interdiscursive character of institutional genres (i.e., genres of the professions). This interest, I suggest, marks a theoretical shift in focus in these fields from the immanent properties of particular texts (or performances) to the processes of intertextuality, interdiscursivity, and recontextualization (Linell, 1998)—that is, the processes through which speech events or texts are produced and mediated through their relationship with prior discourse (see Fairclough, 1992).¹

Related to this theoretical shift is the concept of *genre systems* (or sets) first described by Devitt (1991) in her analysis of tax accountants' work and a few years later elaborated upon by Bazerman (1994b) in his examination of U.S. patent applications. Fairclough (1992) as well conceptualized genre system in a comparable fashion, suggesting that "a society or a particular institution or domain within it has a particular configuration of genres in particular relationships to each other, constituting a system. And of course the configuration and system are open to change" (p. 126). Examples are the systems of texts that distinguish from each other the professions of law, medicine, psychiatry, and so forth, as these professions are constituted in different cultural contexts (e.g., trial law in the United States compared with trial law in India).

The point I wish to make here is that *the professions are organized by genre systems and their work is carried out through genre systems*. The notion of genre system enables us to characterize actors' specific discursive practices in the context of chains of interrelated genres (what Fairclough, 1992, and Linell, 1998, p. 149, called "intertextual chains") that both constitute and are constituted by institutional practices. It should be noted that the concept of genre systems was anticipated in the late 1970s by Latour and Woolgar's ethnography, *Laboratory Life* (1979/1986), which characterized scientific production in terms of networks of interrelated texts that functioned to link actors, laboratories, experimental animals, and equipment in the service of accumulating a field's stock of knowledge or "credit" (capital). In our discussion of the production of scientific knowledge (Berkenkotter & Huckin, 1995), Tom Huckin and I used a flow chart to graphically represent the processes through which lab knowledge, via the production and dissemination of texts, becomes a part of the genre systems linked to the activities of journal peer review and grant seeking. These theoretical and methodological developments in science studies and in rhetorical studies of professional discourse address what has heretofore been a chronic methodological problem in the social sciences: bridging the gap between macrolevel perspectives of

social structure as shaping and determining human activity, on one hand, and microlevel perspectives of situated, everyday practices as constitutive of social structure, on the other (see Bazerman, 1997; Berkenkotter & Huckin, 1995).

The ongoing discussion of the micro-macro split has fostered unnecessary dichotomous thinking. I say this because studying institutional genres requires the investigator to examine the ways in which they *recontextualize* prior texts and discourses. This examination in turn requires that the researcher have considerable knowledge of the particular institutional discursive system under study and that he or she also possess the means for examining actors' situated, meaning-making activities, which are densely contextual (e.g., Beaufort, 2000). This kind of perspectival shift from microlevel (situated activities) to macrolevel (system of texts) has a diachronic dimension as well: intertextual systems of texts, be they patent applications, psychiatric and medical case histories and journal articles, or legal briefs, are documents that are linked to dynamic and historically evolving discursive practices.

In a recent study (Ravotas & Berkenkotter, 1998), Doris Ravotas, a clinical psychologist, and I examined several psychotherapists' written records of psychotherapy sessions for traces of the influence of *nosology*, a genre that has historically been associated with the science of classification—originally of plants and animals—but of diseases as well (see Blashfield, 1984/1990, pp. 77-81, for a discussion of the important differences between biological and psychiatric classifications; see Pichot, 1994 for a history of psychiatric nosology).² Classification is one of the most basic of the human cognitive activities; it involves sorting and transforming various phenomena "into the categories and events that are relevant to the work of [a] profession." (Goodwin, 1994, p. 608) The term nosology refers both to the genre and to the practices of classification as they developed over several centuries. The current incarnation of nosology in the mental health professions is the *Diagnostic and Statistical Manual of Mental Disorders, 4th ed. (DSM-IV)* of the American Psychiatric Association, a text that some commentators call the "psychiatric Bible" (Kutchins & Kirk, 1997; Wylie, 1995).

One way to investigate the influence of nosology, in this case *DSM-IV*, on clinicians' writing, was for Ravotas and I to examine the local, situated writing of several therapists. But we also needed to consider the circulation of the therapists' reports through the mental health system and other more distant institutional systems because

therapists' texts are read and acted upon by any number of mental health professionals, insurance agents, psychiatrists, physicians, auditors, lawyers, judges, and so on. Thus, rather than focusing on a single text or repertoire of texts, we needed to think "system." This methodological exigency led to our conceptualizing institutional speech genres (such as those we were studying) as one of a cluster of interrelated core concepts, including generic intertextuality and interdiscursivity, recontextualization, genre systems, and activity systems.

THEORETICAL FRAMEWORK

I will return the above issues shortly; I need to at this point, however, introduce the theoretical assumptions that have guided this recent research. This list can be appended to the list of 5 principles that appeared in Berkenkotter and Huckin (1995).³

*Genre systems play an intermediate role between institutional structural properties and individual communicative action.*⁴ By the notion of "intermediate role," I mean, following Günthner and Knoblauch (unpublished manuscript; cf., Günthner & Knoblauch, 1995),

Communicative genres [and their repertoires] are not to be separated from the social structural features. They are links between subjective stocks of knowledge and the social structures of a community. Genres fulfill important functions with respect to coping with, transmission and traditionalization of intersubjective experience of the life-world. On one hand they facilitate the transmission of knowledge by guiding the interactants' expectations about what is to be said. On the other hand they are the sediments of socially relevant communicative processes, as only those processes may be expected to be fixed into genres which are of some relevance to the social actors. (p. 6)

From this perspective, genres and their systems can be said to instantiate—through actors' microlevel, situated practices—structures of social and institutional relations. Knowledge of these relations is tacit, imbricated in our symbolic systems and communicative genres—matters of practical knowledge and mutually held by members of a particular profession or discipline. As social/institutional relations across time and space are altered by technology and the changes in an organization's communicative budget, hybrid genres arise in relation to actors' microlevel activity. And here is where one's conceptual framework has to be multidimensional enough to be inclusive of

the following: (a) actor + goal (object), (b) tools (including technology and symbolic systems) and their affordances⁵, (c) constitutive rules, and (d) traces of historical cultural practices (i.e., how these goals have been accomplished in the past).

One of the central means for identifying texts in a genre system is their intertextual activity. The texts that we see in a genre system are responsive to, refer to, index, or anticipate other texts. (*Text* here refers to both spoken, written, and electronically-mediated discourse.) As Fairclough (1992) suggested, "the concept of intertextuality points to the productivity of texts, to how texts can transform prior texts and restructure existing conventions (genres, discourses), to generate new ones" (p. 102).

Intertextuality, as described by Fairclough and others (Bazerman, 1994a, pp. 194-214; Berkenkotter & Huckin, 1995; Bhatia, 1997; Russell, 1997a; Selzer, 1993), is a concept with considerable heuristic or explanatory power because it provides an analytical framework for examining the relationship between diachronically or synchronically linked genres and systems of genres. The concept of intertextuality also enables us to see interrelationships between written and spoken activities in an organizational "communicative chain," as Gunnarsson (1997) recently proposed. Thus, in an engineering consulting firm, a proposal sent to a client is the outcome of many linked genres: oral, written, and electronically-mediated (see Orlikowski & Yates, 1994; Yates, Orlikowski, & Okamura, 1999).

The concept of genre systems enables the analyst to foreground the discursively salient component of human activity systems. Researchers who study intertextually linked writing practices, speaking practices, or both tend to focus either on what the actors are doing ("follow the actors"), such as the dyadic interactions of conversation analysis, or they focus on the features and conventions of texts ("follow the texts"). With these analytical foci, however, it is easy to relegate to the background the many different kinds of contexts in which the situated activity being investigated is occurring—contexts such as the physical setting, the material practices, and socioeconomic structures of the profession or organization; the history of its practices; the participants' background knowledge; the interpersonal relations between the participants (e.g., doctor-patient, therapist-client, nurse-doctor); the cotexts; and so forth (see Y. Engeström, 1999). In fact, the very concept of context is itself problematic, as numerous commentators have observed (see Cole, 1996; Duranti & Goodwin,

1992; Y. Engeström, 1993; Lave, 1988; Linell, 1998; Russell, 1997a; Tracy, 1998). How do we study, analyze, describe, and theorize context? Understanding context as being multidimensional, historically sedimented, cultural, dialogical, discontinuous, contradictory, social, and ideological requires a particular kind of theoretical lens.

To circumvent the problem of treating context either as "containers for actions or as situationally-created experiential spaces" (Y. Engeström, 1993, p. 67), scholars and researchers in a number of disciplines have been using the multidimensional concept of *activity system* in place of context. (See, for example, Cole, 1996; R. Engeström, 1995; Y. Engeström, 1987, 1993; Y. Engeström, Miettinen, & Punamäki, 1999; Lave & Wenger, 1991. For examples of North American writing researchers drawing on activity theory, see Bazerman, 1997; Berkenkotter & Ravotas, 1997; Dias, Freedman, Medway, & Paré, 1998; Freedman & Smart, 1997; Prior, 1997; Russell, 1997a, 1997b; Winsor, 1999.)

Yrjö Engeström's *Learning by Expanding: An Activity-Theoretical Approach to Developmental Research* (1987) and his chapter in the collection, *Understanding Practice* (1993), have been instrumental in introducing the concept of activity system to researchers in the North American tradition of genre studies (Freedman & Medway, 1994; Hyon, 1996) and readers of educational research and theory (see also Cole & Engeström, 1993). Building on neo-Vygotskian activity theory, Engeström (1993) observed,

If we take a closer and prolonged look at any institution, we get a picture of a continuously constructed, collective activity system that is not reducible to series of sums of individual discrete actions, although human agency is necessarily realized in the form of actions [Leont'ev, 1978] (p. 66)

From the perspective of activity theory, contexts should not be considered to be containers nor what Y. Engeström called "situationally-created experiential spaces." Contexts are activity systems.⁶ According to Y. Engeström (1993),

An activity system integrates the subject, the object, and the instruments (material tools as well as signs and symbols) into a unified whole. An activity system incorporates both the object-oriented productive aspect and the person-oriented communicative aspect of the human conduct. Production and communication are inseparable. (p. 67)

Underlying Y. Engeström's (1993) conception of human activity is the view that communicative acts (and their genres) are part and parcel of the subsystems of production, distribution, exchange, and consumption (p. 67; see also Cole, 1996, pp. 139-141, for a discussion of these issues). In this respect, activity theory is a materialist theory. But it is not a deterministic theory in the sense that the life of an activity system is also discontinuous:

Besides accumulation and incremental change, there are crises, upheavals and qualitative transformations. . . . [A]n activity system is not a homogeneous entity. To the contrary it is composed of a multitude of often disparate elements, voices and viewpoints. This multiplicity can be understood in terms of historical layers. An activity system always contains sediments of earlier historical modes. (Y. Engeström, 1993, p. 68)

One way to observe the disparate elements, or dissonances, within activity settings such as banks, nursery school classrooms, or community health care clinics is to examine those settings' genre systems and their constitutive interdiscursive and intertextual practices. By this I mean that the researcher, using some combination of case study or participant-observation techniques, plus fine-grained linguistic/ rhetorical analyses of oral and written texts, seeks to identify the sorts of *multivoicedness* and *inner contradictions* existing within the institutional setting. This is where finding the appropriate unit or units of analysis for examining intertextuality, interdiscursivity, or recontextualization as discursively salient phenomena within the activity system becomes a crucial decision.

There is one last important point to be made regarding activity systems. Y. Engeström (1993; see also Y. Engeström & Miettinen, 1999), following Marx and Leont'ev, suggested that in capitalist societies there exists a basic contradiction between the "use value" and "exchange value" of commodities (see Y. Engeström, 1999); this contradiction exists within the bureaucratic and rationalized medical work activity in which many mental health workers and psychotherapists are engaged. For example, the use value of a psychotherapist's case notes (to that therapist) is tied to planning for future treatment. In contrast, the exchange value of the information in the case notes is tied to a numerical billing code for reimbursement to the clinic or agency for

the therapist's time. The work activity of family practitioners or clinical psychologists reflects a tension between their dual role as "gatekeepers and cost-efficient producers" on one hand, and as "healers or consultants on the other hand." (Y. Engeström, 1993, p. 72).

Having presented the theoretical assumptions grown out of my excursion over the last several years into the arenas of clinical psychology and psychiatry, I now turn to describing a central thematic element (Emerson, Fretz, & Shaw, 1995) in the narrative that my research collaborator, Ravotas, and I constructed from the data we gathered and analyzed. I use the term and concept of *paperwork* to characterize the entire system of texts produced by mental health workers in carrying out the constitutive activities of a community health care agency.

PAPERWORK IN THE ACTIVITY SYSTEM OF A RURAL MENTAL HEALTH CLINIC

The various paperwork genres produced in a medical or mental health clinic coordinate the many different kinds of activity occurring within that setting. For example, in the community mental health center in my rural area of northern Michigan, the constituent activities include outpatient therapy, community support for clients living on their own, care of clients living in group homes, and so forth. To keep track of these activities and also to document them for billing purposes (to Medicaid or insurance companies), mental health workers from therapists to mental health aides (nonprofessional support staff) write numerous reports. Similarly, each action that is undertaken by a mental health aide with regard to the clients living in the group homes (e.g., a trip to town for coffee or a visit to the doctor for a blood draw to check on side effects of a client's medication) is written up. Depending on the kind of activity and its paperwork, these reports circulate from writer to supervisor, to psychiatrist (should medication or a referral be necessary), to case manager, or to other therapists who at some future point will work with the client. As a genre system, these texts serve the institutional function of synchronizing the various kinds of work activity and maintaining an institutional record that may be consulted at some future time.

Recontextualization in a Psychotherapy Interview—and After

One of the many constitutive activities within a mental health clinic is outpatient therapy. Typically, although not always, a client visits a therapist for the first time with the goal of seeking help for dealing with any number of pressing problems. These may be emotional, interpersonal, financial, social, school- or work-related, somatic, or the client may express them in some combination. During an initial 55-minute interview, a therapist talks with the client and, taking notes (or using an audio recording device), gathers information in the form of a history of the client's problems. This the therapist does by asking the client a number of questions that direct the client's talk in particular ways (see Hak, 1992; Hak & deBoer, 1995, 1996; McCarthy, 1991; Young, 1995).

One of the therapist's purposes in this initial session is to determine that certain categories of problem and not others, in the context of the *DSM-IV's* Multiaxial Assessment System, are relevant in this case.⁸ While the client may be describing the effects on him or her of a particular painful or traumatic experience—divorce, sexual abuse, a death in the family—the therapist is determining, from his or her narrative and from the observable symptoms he or she presents, a diagnosis that will guide how she will treat the client and bill a third-party payer for her services. This process of determining which classification of disorder best captures the client's stated problems and his resultant symptoms during an initial interview and which classifications are secondary or not applicable is called *making the differential diagnosis*. Thinking in terms of these categories enables the therapist to make a diagnosis based on the information that the client provides during this first interview. I do not mean to imply here that the therapist does not attend as well to the explicit details of the client's narrative with regard to future therapy sessions or to issues that are salient in the client's comments. Nevertheless, the therapist's use of these categories or classifications constitutes her generic expectations; the information the client provides during the initial interview is filtered through the therapist's highly developed *DSM*-based schema as well as schemata that reflect his or her training in one or more schools of psychotherapy. This process is complicated by the fact, as Capps and Ochs (1995) noted, that

therapists employ differing interpretive principles in their roles as co-authors. Therapeutic theories, whether behavioral, cognitive, humanistic, psycho-dynamic, structural or systemic, contain story lines or master narratives about the locus of psychological disturbances (within the individual or a system) and their appropriate treatment. Well-developed scientifically-based narratives of the human psyche shape the stories initiated by clients. Therapists accentuate certain features of client-initiated narratives, while relegating others to the background. (pp. 176-177)

Capps and Och's observation underscores the complex nature of the institutional conflict between the therapist's dual roles of healer and consultant, on one hand, and gatekeeper and cost-efficient producer, on the other. That is to say, therapists' particular theoretical orientations affect how they listen and what they foreground or background in the session notes they take during the initial interview with a client. At the same time—and largely as a result of the codification of diagnosis attributable to the publication of the *DSM-III* (1980), *DSM-III-R* (1987), and *DSM-IV* (1994) and the increasing bureaucratization of mental health care over the past 25 years—therapists have become increasingly circumscribed in their writing of the initial [psychosocial] assessment, the text that is based on the material in the session notes and that the therapist most often writes shortly after the initial interview.

In writing the initial assessment (also called variously "psychosocial assessment," "screening summary," or "initial intake summary," and so on, depending on the agency), the therapist must translate into psychiatric nomenclature the information the client provided during the initial interview. Unlike the session notes, which are written at the point of the client's utterances, the assessment text represents the therapist's formal evaluation of the client's often multidimensional difficulties, culminating in a diagnosis coded numerically for the purpose of billing third-party payers. The diagnosis is based on the therapist's selecting one from the 410 classifications of psychopathology appearing in *DSM-IV* (1994). In some mental health clinics, the preferred nosology is the *International Classification of Diseases 10* (*ICD-10*) (1978), whose classifications of mental disorders are cross-referenced with those of the *DSM-IV*.

An Illustration

These are a therapist's notes taken during an initial interview with a client. As mentioned above, from these notes the therapist constructs a summary of the client's problems, or the initial assessment, as it is called in the clinic where this particular therapist practiced. This document contains a number of sections, including the reason

for the referral, a description of the client's presenting problem, a narrative of the client's family and personal history, his or her psychiatric and medical history, and an appraisal of the client's mental status. These descriptions provide the foundation for the therapist's diagnostic evaluation of the client, according to the classifications of psychopathology that appear in the *DSM-IV*.

There is much information in this text that is visible. From a discourse-based interview with the therapist, we discovered why she included in quotations a number of the client's comments on the left-hand side; why she circled the comment, "Often feel like I'm not there," and connected this comment with a line to the circled description, "spacy," beside the abbreviated "diss." (for dissociation) on the right-hand side of the page; and why she included such phrases as "Dep. Affect/loose and choppy," "suicidal," "passive now." We also learned her purpose for including the visual diagram on the upper right-hand side of the page. This diagram is a shorthand way of showing family relationships—very important information to this therapist because of her professional training in family therapy. With the help of the therapist as a specialist informant, it was also possible to learn her purpose for including the three diagnostic descriptors, PTSD (post-traumatic stress disorder), depression, and psychosis, each followed by question marks.

In examining the graphic inscriptions in this text—i.e., the lines connecting circled utterances, the schematic for showing family relationships, and the dividing of the page into different sections—it is possible to see the therapist drawing on different kinds of background knowledge. One can also discern traces of *DSM-IV*'s shaping activity on the therapist's inscriptions. As she takes notes on what the client is saying, she is making inferences from the client's comments that will lead to the diagnosis that appears at the end of the Initial Assessment.

A casual perusal of Figure 1, however, does not reveal its multivoicedness, that is, the other texts and utterances to which these notes are connected. These texts include the oral interaction between client and therapist during which these notes were composed, the written evaluation of the client's mental status, i.e., the initial assessment that the therapist later constructed from the notes, and the other documents that characteristically follow the assessment in a sequence of texts that report the client's progress over the course of therapy. We do not see the system of genres within the mental health agency to which these texts belong, nor can we discern other textual networks in

other discursive systems (e.g., actuarial, legal, and medical) with which this therapist's written accounts may be linked at some future time.

To consider these contexts, imagine, if you will, threads linking the notes in Figure 1 to a genre system including the therapist's initial assessment, treatment plan, progress or process notes, and termination summary. Imagine those texts within the cycle of oral and written genres that constitute the activity of psychotherapy and, beyond that activity, as documents that make up the communicative economy of the mental health care agency in which the aforementioned therapist practices. Finally, imagine the information in the session notes recontextualized in the initial assessment in the context of other institutional systems (e.g., legal, medical, and actuarial) to which an assessment text (and others like it) might travel.

Engaging in this kind of thought experiment necessitates developing an interpretive framework capacious enough to enable the researcher to conceptualize Figure 1 as a site of discursive practices within multiple institutional genre systems. The concept of *multiple activity systems* makes it possible to consider both the local settings of the mental health clinic with its salient activities and practices and the more distant sites to which documents (or parts of documents) from the clinic might travel and the objects or activities within those sites that shape how therapists report information in their written records. And the concept of *institutional conflict*, or dissonance (Y. Engeström, 1993), enables us to reflect on how a therapist's dichotomous roles of healer and gatekeeper are instantiated in the session notes and initial assessment.

The therapist sees her activity from a perspective located in the ethnographic present. Although she may be quite reflexive about what she is doing in the notes shown in Figure 1, she may not possess (unless she is a student of the history of psychiatry) a historical understanding of the sources of her textual practices. Her discursive knowledge (Giddens, 1984), sophisticated as it may be, may not include a diachronic awareness of the suasive force of nosology—as a genre or as a set of historically sedimented practices—that informs her note-taking activity. And she may not perceive the ways in which the *DSM* functions to link the social worlds of therapist-practitioner, psychiatrist, physician, social worker, insurance company auditor, and lawyer with that of the client. However, she may well understand, as did other therapists we interviewed, that a client's identity comes to be constituted through the therapist's written texts, texts that are read by

other therapists, the therapist's supervisor, the psychiatrist or psychiatrists at the agency, an auditor, a judge, a new therapist, or an insurance company auditor.

It is this nosology's semantic resources, specifically its standardized classifications, that enable the therapist to communicate with the representatives of these social worlds. A genre such as the *DSM-IV* or the *ICD-10*, and the constellation of texts that surround it, functions as a genre system to link multiple social/professional/institutional worlds. Such systems are an essential part of the communicative economies of various professions (and disciplines and other collectives); their use establishes continuity of information crossing professional boundaries, despite the different interpretations and uses of that information.

COMMENTARY

Five of the issues I have raised thus far require further elaboration. First, the *DSM-IV* (and its predecessors, *DSM-III* and *DSM-III-R*) can be seen to function as *meta-genres* around which a constellation of professional activities (and their genres) are organized.¹⁰ In this respect, it has become a mediational means or tool for stabilizing practices, such as conducting research (if one is a research psychiatrist or psychologist), diagnosing conditions, prescribing medication, writing treatment plans, and so forth. As a classification system, the *DSM-IV* functions to standardize mental health practitioners' work activities toward the object or end of making consistent diagnoses across professional boundaries.

Second, as suggested above, psychiatrists, psychologists, and social workers who make up the practitioner ranks of psychotherapists have vastly different kinds of training and orientation: They speak many different disciplinary or professional languages depending on whether their clinical orientation has been psychoanalysis, behaviorism, Rogerian psychology, family systems, and so forth. In contrast, by using the various classifications of psychopathology, as they appear in the latest *DSM*, the *DSM-IV*, the mental health professional draws upon a common lexicon that has been developed by research psychiatrists ascribing to a medical model. So, although the practitioner work of psychiatrists and clinician therapists is heterogeneous, psychiatric research is based on creating common understandings, ensuring reliability across domains, and gathering information

in a way that retains its integrity across time, space, and local contingencies. This set of conditions results in a central tension within the psychotherapy community, a tension between the heterogeneity of different and often multiple therapeutic orientations and the need for reliability and generalizability of diagnosis. In its use of the classifications of the *DSM-IV* for diagnosis, although not necessarily as a basis for therapy, psychotherapy paperwork reflects the often contradictory practices of codification and heterogeneity.

Third, some sense of the *DSM-IV*'s suasive force can be seen in the large number of books in various genres that are published to help the practitioner use the *DSM-IV*. A search in the fall of 1997 turned up 79 titles of companion volumes in the form of how-to books, textbooks, scholarly collections, guides, or users' manuals bearing such titles as *Disorders of Personality: DSM-IV and Beyond* (Millon, 1995); *The First Interview: Revised for DSM-IV* (Morrison, 1994); *DSM Casebook: A Learning Companion to the Diagnostic and Statistical Manual of Mental Disorders* (Spitzer, Gibbon, Skodol, & First, 1994); *Interview Guide for Evaluating DSM-IV Psychiatric Disorders* (Zimmerman, 1994); *Personality Disorders—Clinical and Social Perspectives: Assessment and Treatment Based on DSM-IV and ICD-10* (Derksen, 1995); and *Study Guide to DSM-IV* (Fauman, 1994), to name a few.

Not the least of these filiations within the *DSM-IV* system is a sequence of texts, the *DSM-IV Sourcebook*, volumes 1 to 3 (Widiger, et al., 1994, 1995, 1996), which contain literature reviews of the research conducted on the various broad categories of classifications, such as the schizophrenias, the affective disorders, the personality disorders, and so forth. Unlike the scientific article, in which the authors use citations variously as warrants or backing for their knowledge claims, the construct validity and reliability of the *DSM-IV* classifications have been established by the literature reviews appearing in the *Sourcebook* series. At least this is the claim made by the editors of the *Sourcebook* series (1994, pp. xviii-xix). It remains to be seen whether the literature reviews appearing in these volumes will provide the same kind of suasive force that citation practices accomplish in the scientific paper.

Fourth, although architects of *DSM-III*, *DSM-III-R*, and *DSM-IV*, such as Robert Spitzer, Allen Francis, and Harold Pincus, insist that the *DSM* classifications represent research-based psychiatric classifications (see Francis, Pincus, Widiger, Davis, & First, 1990), there exist many accounts of the political and social factors surrounding the adoption or rejection of various classifications. Probably the most

well-known of these was the effort to omit the classification of "homosexuality" as a disorder in *DSM-III* (see Blashfield, 1984/1990; Kutchins & Kirk, 1997; Spitzer, 1973). But there have also been organized campaigns either for or against the American Psychiatric Association's adoption of other classifications: e.g., post-traumatic stress disorder (Young, 1995), post-luteal phase disorder (Figert, 1996), and self-defeating personality disorder (Kutchins & Kirk, 1997; Caplan, 1995).

Finally, the use of classification systems, according to Goodwin (1994), entrains the user into what he describes as "professional vision" (see also Goodwin, 2000). Goodwin (1994) suggested that "central to the social and cognitive organization of a profession is its ability to shape events in the domain of its scrutiny into the phenomenal objects around which the discourse of the profession is organized" (p. 626). Although different professional groups have access to different theories, artifacts, bodies of expertise, and therefore, different discursive practices, these practices are homologous in the sense that they entail (1) coding, which transforms phenomena observed in a specific setting into the objects of knowledge that animate the discourse of a profession; (2) highlighting, which makes specific phenomena in a complex perceptual field salient by marking them in some fashion; and (3) "producing and articulating material representations" (Goodwin, 1994, p. 606).

Sarangi (1996) noted that "Goodwin's observation goes beyond the 'what' aspects of professional knowledge, as he offers insights into 'how' professional knowledge claims are legitimated and contested through 'ways of seeing'" (pp. 2-3). In the context of a mental health clinic, professional vision operates as an *archetypal professional practice* accomplishing the institutional goal of selective information management through the deployment of "procedural criteria which underlie the very existence and transformation of institutional categories" (Sarangi, 1996, p. 3). The psychotherapist's practice of making notes and reports that recontextualize the individual's self-reports and interactions within psychiatric discourse begins the work of drawing the individual clients into the systems of reimbursement, health care, research, and medical reasoning. Perhaps even more important, psychotherapy notes and reports are the site at which we see the therapist constructing accounts that may then influence how the clients themselves may begin to recontextualize their own perceptions of themselves (Berkenkotter & Ravotas, 1997, p. 257).

Most clients come to therapy with fuzzy problem sets—i.e., a cluster of heterogeneous, multidimensional problems, often presented in narrative form. The activity of placing a coding scheme like a grid over the problems and life experiences that clients bring to therapy is what researchers have variously described as “transformation,” “(re)formulation,” “(re)classification,” “institutional framing,” or “bureaucratic interpretation” (Sarangi, 1996, pp. 242-243). This activity functions to reify the client as his or her disorder, with the consequence of reducing that person’s identity to what psychiatrists call the “clinical picture.” In effect, the client becomes the sum of his or her presenting symptoms. In this fashion, the actions and events within the client’s narratives are recontextualized through the highly nominalized grammar/lexicon of the therapist’s representation of the client’s problems in living.

CONCLUSION

In this essay, I have used genre theory—with its most recent embellishments from activity theory—as a lens for examining the influences that bear on the paperwork practices of mental health workers in a rural mental health clinic. In this sense, the article uses case study material to illustrate theoretical statements or claims. The kind of micro-macro level perspective shifting I have described above requires that the unit of analysis be microlevel, but that the conceptual framework enable the researcher to take a longer historical view, one that allows her to see people’s activities embedded within traditions of activities, texts, institutions, and organizations. In this respect, the essay itself has been an exercise in localization, moving from the largest notions of activity or genre system to the specific historically evolved system of nosology and related practices, to the specific institutional system of one clinic, to the microinteractions and characterizations and representations emanating from one therapist-client interaction during one 55-minute period in one office.

The ways in which professional vision is accomplished in a therapist’s session notes (a local act that is linked to other sites of representational activity) was central to our understanding of the relationship between the macrolevel and microlevel dimensions of activity systems. To conceptualize the influence of nosology on clinicians’ discursive practices, my research collaborator, Doris Ravotas, and I needed the methodological means for being able to look at local, situated activity. At the same time, we needed to consider (conceptually) the

professional discursive systems through which the therapists' texts were likely to travel. Only then could we begin to grasp the functions of nosology (as practice and genre) in large networks of actors and texts.

There is a further need in genre studies to develop methods for examining the situated knowledge of the users of genres in the context of historical practices as these practices have been transmitted by genres over time. Context, given the heterogeneous activity that can be observed in the therapist's notes in Figure 1, required being examined from a sociocultural perspective. Studying context also involved bringing together a macrolevel theoretical interpretive framework with microlevel data we could identify in the therapists' records.¹¹

Many writers, among them applied linguists, anthropological linguists, ethnomethodologists, and literary theorists have recently addressed the issue of methodology for studying relationships between text and context (Duranti & Goodwin, 1992; Tracy, 1998; Watson & Seiler, 1992). Some have raised the question of whether or not it is heuristically useful to use this framework because of the "container/thing contained" metaphorical associations that the notion text/context evokes (Russell, 1997a). Rather than enter what appears to me to be an academic debate, I would prefer to suggest that concepts such as genre systems, intertextuality, and interdiscursivity are useful tools for understanding complex, historically mediated text/context relationships. Those of us who study genres in many different kinds of settings can learn much, methodologically speaking, from a cultural/historical approach to studying situated language activities. The concept of context as activity system is multidimensional, thus lending itself to a variety of interdisciplinary approaches and methods.

NOTES

1. In *Text*, 18(2), 1998, a special issue on the topic of "discourse across professional boundaries," editor Per Linell's analytic focus was on recontextualization practices in professional discourse settings, such as social work, geriatric medicine, and mental health care. Using a broad typology, Linell suggested that professional discourse could be seen as falling under three categories: *intraprofessional*, *interprofessional*, and *practitioner-client interactions*. As described by Linell (1998), *recontextualization* refers to

the dynamic transfer-and-transformation of something from one discourse/text-in-context (the context being in reality a matrix or field of contexts) to another. Recontextualization involves the extrication of some part or aspect

from a text or discourse, or from a genre of texts or discourses, and the fitting of this part or aspect into another context, i.e., another text, or discourse (or discourse genre) and its use or environment. . . . Aspects of discourse which may be recontextualized include linguistic expressions, concepts and propositions, 'facts' arguments and lines of argumentation, stories, assessments, values and ideologies, knowledge and theoretical constructs, ways of seeing things and ways of acting toward them, ways of thinking, and ways of saying things. (pp. 144-15)

2. *Nosology*, or the classification of diseases, evolved from botanical taxonomy, which was originally a branch of medicine. Thomas Sydenham in the 17th century and Carl von Linnaeus in the 18th century were among those who attempted to apply methods of botanical classification to medicine. Pichot (1994), in a historical survey of nosological models in psychiatry, noted "from [Sydenham] on disease became a composite term describing the common features of a variety of individual cases, and nosology, a branch of medicine concerned with their definition" (p. 232). For a "textography" of the genres of botanical taxonomy and the discursive practices of four taxonomists at the University of Michigan's Herbarium, see Swales (1998a).

3. These five principles are as follows (for a detailed discussion and examples of this material, see Berkenkotter & Huckin, 1995, pp. 1-25):

Dynamism. Genres are dynamic rhetorical forms that are developed from actors' responses to recurrent situations and that serve to stabilize experience and give it coherence and meaning. Genres change over time in response to their users' sociocognitive needs.

Situatedness. Our knowledge of genres is derived from and embedded in our participation in the communicative activities of daily and professional life. As such, genre knowledge is a form of "situated cognition," that continues to develop as we participate in the activities of the ambient culture.

Form and content. Genre knowledge embraces both form and content, including a sense of what content is appropriate to a particular purpose in a particular situation at a particular point in time.

Duality of structure. As we draw on genre rules to engage in professional activities, we *constitute* social structures (in professional, institutional, and organizational contexts) and simultaneously *reproduce* these structures.

Community ownership. Genre conventions signal a discourse community's norms, epistemology, ideology, and social ontology.

4. Compare, Miller (1994) and Swales (1993).

5. Rogoff (1990) and others have adapted Gibson's (1979) ecological theory, which emphasizes the mutuality of environment and organism; features of animals and environments each fit with the characteristics of the other, in the sense that an animal that occupies a particular ecological niche (including its medium, predators, prey, nest, etc.), can be described in terms of that niche vis à vis structure and function. In the context of activity theory, a particular object, such as a stick or a chisel, has the potential to be grasped by a particular animal, and conversely, that animal (or animals) possesses the potential to grasp that object (Rogoff, 1990, pp. 30-31). The notion of affordances is particularly rich when we consider the affordances of symbolic tools, such as sign language, and the potential of certain animals and humans to use them to communicate.

6. This approach may be open to criticism for being “too broad” or “too abstract” to do justice to the complex ways in which speakers and writers instantiate and generate context on a moment-by-moment basis. In considering context as an analytic concept, see also *Research on Language and Social Interaction*, 31(1), 1998, for a discussion among discourse analysts of the myriad approaches to context that exist within this community. Specifically, their concerns were with “the use of context in the analysis: the meaning of the term and the kinds of context each researcher [in the issue] brought to his or her data (and why)” (Tracy, 1998, p. 2). Especially interesting are the approaches to context described by Swales (1998b, pp. 109-122) and the summary by Pomerantz (1998, pp. 123-132), who noted that conceptualizing context as a resource that participants draw on in interaction is a meaningful alternative to treating context as shaping or influencing conduct.

7. To further elaborate on this distinction between use and exchange values of commodities, Y. Engeström (1999) noted, “Marx’s concept of labor or the production of use values was the paradigmatic model of human object-oriented activity for Leont’ev when he formulated the concept of activity” (pp. 3-4). But it is also important to keep in mind, Engeström added, that Marx’s analysis of capitalism includes, “above all the concept of commodity as a contradictory unity of use value and exchange value. This dialectical concept is crucial for any serious analysis of the contradictory motives of human activities and human psyche in capitalist society” (p. 5).

8. *The Diagnostic and Statistical Manual of Mental Disorders*, 4th ed. (DSM-IV’s) multiaxial system is putatively designed to help psychotherapists and other diagnosticians evaluate the client or patient from several different analytical frames: Axis I, Clinical Disorders (e.g., schizophrenic disorder); Axis II, Personality Disorders, Mental Retardation; Axis III, General Medical Conditions; Axis IV, Psychosocial and Environmental Problems; Axis V, Global Assessment of Functioning. According to the DSM-IV’s authors, who are not specifically listed, such a system provides a “convenient format for organizing and communicating clinical information, for capturing the complexity of the clinical situation, and for describing the heterogeneity of individuals presenting with the same diagnosis” (1994, p. 25). The introduction of a multiaxial system to DSM-III and its successors is designed to promote the application of a biopsychosocial model in clinical, educational, and research settings. However, as many of our respondents told us, only diagnoses based on Axis I and II classifications are reimbursable.

9. Doris Ravotas and I tracked and analyzed the therapist’s processes of recontextualizing client information within and between the session notes, described above, and the institutional report (initial assessment) constructed from the notes. Using a coding scheme that provided us with categories of reported speech, Ravotas coded both the session notes and the initial assessment to determine the syntactic, grammatical and lexical devices the therapist used to represent the client’s utterances in both texts (Ravotas & Berkenkotter, 1998). This analytic procedure provided us with a way of operationalizing the concept of recontextualization.

10. I would like to thank Janet Giltrow (1998) for coining this term. I use it here to refer to commodious genres around which other genres cluster and from which they draw features, conventions, and substance (content). Metagenres are mutable and possess resources for translating information across professional and cultural boundaries. Compare McCarthy’s (1991) use of the term “charter document” in describing the function of DSM-III. She observed that as a charter, DSM-III operated to “authorize certain ways of seeing, deflecting attention from other ways, and stabilizing the biomedical conceptual model of mental illness” (p. 359). Although acknowledging McCarthy’s

insightful choice of that metaphor to characterize the performative power of *DSM-III*, my interest is more in *DSM-IV*'s capabilities as a powerful *networker* (Latour, 1987). Consequently, I have employed the concept "metagenre" to capture *DSM-IV*'s ability (as a nosology) to create and sustain networks for transporting knowledge across various contexts, disentangling it from local practices.

11. These data are described in detail in two recent papers (Berkenkotter & Ravotas, 1997; Ravotas & Berkenkotter, 1998).

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